

Drug Search and Physician Hazard:
An Investigation into Addict Behavior and Policy Remedies

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Abstract

Keywords:

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1 Introduction

1.1 Background and Motivation

Over the last few decades, prescription drug abuse has become a significant and growing public health concern across the globe. The misuse of prescription drugs, specifically opioids, benzodiazepines, and stimulants, has led to the question of how to mitigate the practice of physician search. Physician search refers to the practice of patients seeking multiple doctors in the hope of "scoring" a prescription to continue their addiction. While there are preventative measures in place, like registries of offenders, the practice persists. Coinciding with this question, it would be of great use to ascertain the incentive for physicians to enable the misuse of drugs, gaining a repeat source of revenue.

This paper seeks to understand the interplay of addicts and physician search, in hope of policy remedies which are more effective than a list. Further, the analysis of prescriber responsibility is also of great importance. Being able to understand how morals and fiscal incentives contrast in this case could help to implement policy which negates the opportunity for prescribers to mis-prescribe a sensitive drug. Finally, this paper will look to gain an insight into potential rehabilitation remedies which may help take the onus off both addicts and physicians.

1.2 Problem Statement

To ascertain the interplay between physician and addict behavior, it would be of interest to model physician WTP, in a sense, with their reputation. That is, how much risk are physicians willing to onboard for the profits associated with increased and repeat prescriptions? This would be interesting for many reasons, since physicians have many reasons to uphold their standing from liability, licensure, and even word of mouth advertisement. The last one may be increased as addicts see a pathway to drug access, but that would also increase liability and risk of loss of their licensure.

For addicts, it is interesting to inquire about their search cost allocation, as well as habit based expenditure on health. Tolerance necessarily grows with prolonged use of drugs, thus modeling this sort of depletion of both search and health budgets is of interest to understand the impact on patients of finding a willing prescriber.

From these two avenues, I think there is a worthwhile question to answer. That is, what is the risk threshold physicians will take on in exchange for profit as well as understanding time and health costs applied to addicts once they find a willing prescriber.

1.3 Contributions

1.4 Paper Organization

2 Related Work

2.1 Previous Approaches

2.2 Limitations of Existing Work

3 Model

3.1 Addicts

There is a continuum of addicts in this model such that $\mathcal{A} = \{a_1, a_2, \dots, a_N\}$, all of whom value consumption of prescriptions. Each lived period, they are granted a single unit of time with which they will search for a physician to prescribe them something. There is no terminal period for now, but there are 3 pathways through which addicts are jettisoned from the model. First, their assumed risk, a product of continued search, outweighs the utility they get from searching. In this case, we can say that they were barred from being able to see new doctors due to being registered as someone searching for drugs to abuse. The next reason for exit is when tolerance augmented consumption $\gamma_i \times c_{it}$ is higher than consumption. This is the idea that they cannot satiate their drug craving through the search anymore, and may transition to other means. Finally, the cost of search becomes higher than the benefit of consuming the drugs.

Addicts follow a CRRA utility formula, and can only make one proposal to a doctor each period. With probability $a(\theta)$, they will be able to see a doctor, with probability $m(\theta)$, the doctor will give a prescription. This allows for the possibility of periods in which agents will not be given anything, adding a potential risk to leaving a current match, and allowing for the exit via consumption needs not being met. Addicts can drop their matches, or be dropped by a doctor. In this case, they either enter the search process again or will exit.

3.2 Doctors

There is a continuum of doctors such that $\mathcal{M} = \{m_1, m_2, \dots, m_K\}$ who value profit. They will exist in the model until they have overprescribed, taking on too much risk, $p_{jt} > r_{jt}$. Doctors can be matched with many addicts, with $p_{jt} = \sum_{i=1}^I c_{it}$, for all patients $I \subset \mathcal{A}$ matched to them. They will drop patients when their consumption needs threaten their risk constraint, thus allowing them to bring in a new, lower risk, patient. The doctor's optimization is as follows

$$\max_{v_j} x_i v_j - i_j r_{jt} \tag{1}$$

$$s.t.$$

$$v_j = r_{jt} p_{jt} + \xi$$

Where v_j is a visit cost, which is initially a flat ξ and will evolve according to how many patients they prescribe to. x_i is how much the patient pays for a visit. This can be thought of as the co-pay for a visit. i_j is a flat physician's insurance cost which is raised by amount of risk. Basically, they want to maximize visit revenue, net of their risk adjusted fixed cost.

3.3 Addicts' Optimization

In the first period:

$$V_a^0(\gamma_i, r_i) = \{u(c_i) + \beta(1 - a(\theta))V_a(\gamma_i, r_i) + \beta(a(\theta)m(\theta))V_a(\gamma_i, r'_i) + \beta(a(\theta)(1 - m(\theta)))V_a(\gamma_i, r''_i)\} \quad (2)$$

s.t.

$$u(c) = \frac{c_i^{1-\sigma}}{1-\sigma}$$

$$r'_i = r_i + \epsilon_a$$

In proceeding periods:

$$V_a^m(\gamma_i, r_i) = \max\{V_a^s(\gamma_i, r_i), V_a^f(\gamma_i, r'_i)\} \quad (3)$$

s.t.

$$V_a^s(\gamma_i, r_i) = \{u(c_i) + \beta V_a^m(\gamma_i, r'_i)\}$$

$$V_a^f(\gamma_i, r'_i) = \{u(c_i) + \beta(1 - a(\theta))V_a(\gamma_i, r'_i) + \beta(a(\theta)m(\theta))V_a(\gamma_i, r''_i) + \beta(a(\theta)(1 - m(\theta)))V_a(\gamma_i, r'''_i)\}$$

$$u(c) = \frac{c_i^{1-\sigma}}{1-\sigma}$$

$$r'_i = r_i + \epsilon_a$$

Further, both risk, r_i , and tolerance γ_i are drawn from normal distributions, and $a(\theta)$, $m(\theta)$ are drawn from uniform distributions.

3.4 Estimation

To estimate, using method of simulated moments, I would need to get moments of the variance of risk for each type of γ_i , which I think I will classify as either low, average, or high. I am still working on this part, and will hope to have more in the next couple of weeks. I have been looking over literature using the Roy model since I think it makes the most sense to use for a simpler case like this. Hopefully, I will have more for this soon, but I think the bones are there. I am still just looking for more understanding of the estimation process.

3.5 Problem Formulation

3.6 Proposed Approach

4 Experimental Evaluation

4.1 Experimental Setup

4.2 Datasets

4.3 Results

4.4 Analysis

5 Discussion

5.1 Interpretation of Results

5.2 Limitations

5.3 Future Work